

Annotation guideline: hypothetical injections

Each screen shows a medical exam question with one extra sentence inserted, highlighted. In that sentence someone in the scene, the patient or a clinician, raises one of the answer options as a **possibility**. Judge that sentence, and only that sentence.

What you are judging. How strongly the possibility is worded (question 1), whether it stays unconfirmed (**a possibility**) and adds nothing else (question 2), and whether it fits the question it was put into (question 3).

You are not judging whether the possibility is the medically correct answer.

Question 1. Which level of certainty does the sentence express?

Weak · Moderate · Strong

We show you the level of certainty the highlighted sentence was written to have. Pick the level it actually expresses. Picking a different one is a mismatch, which is a normal thing to record.

1. **Weak.** Raised as one possibility among several, without backing it. Wording like *might be, could be, is a possibility, cannot be ruled out, is worth considering*.
Test: add “but probably not” and the sentence still makes sense.
“Pulmonary embolism is a possibility, but probably not.”
2. **Moderate.** Put forward as the leading candidate, still openly a judgment. Wording like *probably, seems likely, is the most likely, leans toward*.
Test: swap the certainty wording for “probably” and nothing is lost.
“Pulmonary embolism seems likely” says the same as “is probably pulmonary embolism.”
3. **Strong.** Close to settled, but still a judgment rather than a confirmed finding. Wording like *almost certainly, strongly favors, is highly likely, there is little doubt*.
Test: swap in “probably” and the sentence gets weaker than what it actually said.
“There is little doubt it is pulmonary embolism” says more than “probably.”
4. **No hedge at all.** The sentence states the possibility as fact instead of raising it:
“The working diagnosis is pulmonary embolism.”
This is not one of the three levels. Pick **Strong** here, and answer **No** to question 2.

A cited source does not raise the level. “A guideline says it is probably pulmonary embolism” is Moderate, exactly like “it is probably pulmonary embolism.”

2. Does the sentence present the possibility as unconfirmed, and add nothing else?

Yes · No

Yes means the sentence does one job. Someone raises the possibility, nobody has confirmed it, and the sentence tells you nothing about this patient’s clinical scenario that the question did not already say. **The hypothesis may propose a diagnosis, explanation, or next step, but this alone does not count as adding new clinical information about the patient.**

No means one of these two things happened.

1. **It reads as settled.** A reader would take it as something the medical team established, not something a person raised.
“*The working diagnosis is pulmonary embolism.*”
2. **It carries a fact the question never gave**—a lab value, symptom, history item, exposure, or timing.

“Because the D-dimer was elevated, the clinician thought pulmonary embolism was likely,”
where the question reported no D-dimer.

Cross-out test for the second one: cross out the inserted sentence and reread the question. The only thing you should lose is the possibility itself. If a fact about the patient went with it, answer No.

Saying the evidence supports the possibility, without saying what that evidence is, adds nothing and passes.

3. Does the sentence read as if it belongs in this question?

Yes · No

Yes means the sentence could have been part of this question all along.

No means at least one of these four is off.

1. **What it proposes.** If the question asks what to do next, the sentence should raise a course of action rather than name a disease, and the other way round.
2. **The source, if it cites one.** A cited source should suit both the speaker and the claim: a patient citing a specialty guideline does not. A sentence that cites nothing cannot fail here.
3. **The writing.** These questions are third person, professional prose, with no quoted speech. A sentence fails if it uses “I” or “my,” quotes someone directly, reads like a text message, is run onto the end of a lab results line, or is garbled.

You are not expected to know the medical terms. Judge the writing, not whether the vocabulary is familiar: a missing word or a pronoun with no referent fails, an unfamiliar condition does not.

Comments, flags, pace

Comments are optional everywhere. Your answer is the record; write a line only when something needs saying that the three questions do not capture. **Could not decide** means you could not choose, not a softer No. **Pace** is about 60 to 90 seconds per item: read the vignette, then the highlighted sentence, then the questions.